

Bisphosphonates for breast cancer

Bisphosphonates are drugs that help to protect the bones against some effects of breast cancer treatment. They can also help reduce the risk of breast cancer coming back after treatment.

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What are bisphosphonates?

Bisphosphonates are drugs that may be used in early [breast cancer](#) to:

- help protect from bone thinning ([osteoporosis](#)), a side effect of some treatments
- reduce the risk of breast cancer coming back – these are called adjuvant bisphosphonates.

Bone protection

Some [hormonal therapies drugs](#) can affect the bones. This is because they cause lower oestrogen levels. Some hormonal therapy and [chemotherapy](#) drugs can cause an [early menopause](#). This increases the risk of osteoporosis.

Your cancer doctor may prescribe bisphosphonates if you are at risk of osteoporosis or already have it. They will usually also advise you to take calcium and vitamin D supplements to help strengthen your bones.

There are lifestyle changes you can make that will help [look after your bones](#). These include:

- eating healthily
- doing regular weight-bearing exercise, such as walking
- not smoking.

Adjuvant bisphosphonates

If you have early breast cancer, bisphosphonates can help lower the risk of the cancer coming back. This is called adjuvant treatment. You will usually have bisphosphonates for 3 to 5 years.

You usually have this treatment if you have a higher risk of the cancer coming back, and you have:

- been through the menopause
- had treatment to stop your ovaries working – for example, [goserelin](#).

The bisphosphonate drugs most commonly used in adjuvant treatment are:

- [zoledronic acid](#), which is given by a drip into a vein (an infusion) every 6 months
- [clodronate](#), which is taken as a tablet
- [ibandronic acid](#), which is taken as a tablet.

Your cancer doctor can tell you if adjuvant bisphosphonates may be helpful for you. It is important to consider the possible benefits against the side effects of the drugs.

Side effects of bisphosphonates

Side effects will depend on the type of [bisphosphonate](#) you have. Some common side effects are:

- mild sickness
- indigestion
- flu-like symptoms.

Bisphosphonates do not usually cause serious side effects. But rarely they can cause problems with the jawbone. This is called osteonecrosis. It can cause the bone in the area to weaken and die.

Taking good care of your teeth and gums can reduce the risk of jaw problems. It is important to have a dental check-up before starting bisphosphonates. Tell your dentist and your cancer doctor or nurse if you have any problems with your mouth.

Rarely, bisphosphonates can affect the kidneys. Your doctor will check your blood regularly to check how your kidneys are working.

About our information

References



Below is a sample of the sources used in our breast cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

ESMO. Early breast cancer clinical practice guidelines for diagnosis, treatment and follow-up. 2019, Vol 30, pp1192–1220. Available from: <https://www.esmo.org/guidelines/guidelines-by-topic/breast-cancer/early-breast-cancer> [accessed 2023].

National Institute for Health and Care Excellence (NICE). Early and locally advanced breast cancer: diagnosis and management. 2018. Updated 2023. Available from: <https://www.nice.org.uk/guidance/ng101> [accessed 2023].

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by [Dr Rebecca Roylance](#), Consultant Medical Oncologist and [Professor Mike Dixon](#), Professor of Surgery and Consultant Breast Surgeon.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 October 2023 | Next review: 01 October 2026

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)